

PRE-SEND CHECKLIST — the psychiatric instruction

■■ **INTERNAL.** This page never goes to the practice, to the employer, or to the Regulator. Everything else in the pack is written to be produced; this is not.

BEFORE ANYTHING IS SENT

0. ■■■ THE DRAFT. Part D asks to be provided with a draft to identify factual error. **When it arrives, the review is FACTS ONLY.** ■ Do not comment on any opinion, do not suggest a conclusion, do not ask for anything to be strengthened — the letter promises it and the promise is the protection. Check: dates · names · which document said what · attachment references · whether every document reviewed is listed · whether it states which Part B sources the causation opinion rests on · whether it uses “*unfair dismissal*” or “*bullying*” · whether the “*not for medico-legal use*” footer appears.

1. ■ FUNDING IS CLOSED — MSH contacted the practice directly and confirmed it will meet the cost. ■ That is the strongest form it could take: not a promise to Cory but a commitment made directly to a third party, of which the practice holds the record. Two small follow-ups remain, and **neither involves contacting MSH:**

■ **(a) The quantum.** “*Will meet the cost*” is not “*will meet the cost up to \$X*”. Confirm the practice's figure with the practice when booking. The PsychGroup benchmark on file is \$10,000–\$30,000 for a full medico-legal report; this instruction asks a narrower question and should be priced well below it.

■■ **(b) What ELSE did the Health Service say to the practice?** If it framed a scope, or sent its own questions, the doctor may believe he is answering MSH's brief rather than this instruction. **Ask the practice one neutral question when booking: what has the Health Service provided to you, or asked of you?** ■ The letter of instruction must be the only instruction. ■ And question 1.5 catches it either way — anything MSH sent will appear in the list of documents reviewed.

1a. ■■ THE ONE THING TO WATCH FOR ON THE DRAFT — HIS OWN 2025 REPORT CODED THE PREPARATION AS A SYMPTOM. The report of 13 February 2025 lists, **under symptoms:** “*obsessive rumination, fear driven thoughts, extensive researching, and planning on fact findings for work-related issues including reading up on policies, procedures, and legislations*”, and records that the proceedings “*have consumed his whole life severely affecting his functioning.*”

⇒ ■■■ **A seven-page forensic instruction, a schedule of assumed facts and five attachments, prepared personally by the patient, is on its face the very behaviour that clinician has already recorded as pathology.** ■ Watch for any sentence in the new report that treats the preparation of the case as a symptom, or the litigation as the barrier to return. **It is an incapacity marker, it is directly usable by the employer, and s 32(5)(c) excludes injury from action taken in connection with the compensation application.** ■ The covering email now states that he is self-represented and prepared the instruction himself, which explains the authorship as a necessity of his position rather than as a symptom.

2. The practice footer. The report of 13 February 2025 carries “*disclosed for Qsuper and not for medico-legal use*” on every page. ■ **If that appears on this report it is compromised on its face.** The covering email raises it; confirm it has been understood.

3. Insert the date in the instruction where marked.

4. ■■ The commencement date at question 1.3(c) is now “since 2019” — more than five years of continuous shift work before the pleaded onset. ■ **Have a document for it** (first payslip, letter of appointment, or service record) before the answer is relied on in the appeal. ■ **The casual-to-full-time change stays out of the letter** — it is a contractual question, not a clinical one, and it has already been run and answered.

5. ■ The capacity wording is VERIFIED and locked — read from the Employee Capability Checklist of 3 July 2026. ■ The old paraphrase “*continuation of existing arrangement*” **does not appear in the document** and has been removed everywhere. The correct words are “*approximately six shifts per fortnight ... the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months*”

without deterioration ... Usual switchboard operational duties remain suitable."

6. ■ **Say nothing about which exhibit holds what, or about the span of the production.** The letter and the schedule state only what the respondent asserts — a history from 26 October 2022 — and nothing about what the file does or does not contain. **Do not add it back in correspondence.** It is their assertion to prove.

STILL MISSING FROM THE PACK — AND WHY IT MATTERS

7. ■ **The three movement forms are IN the pack** — Attachment 5, stitched with an index page: 27.02.2026 · 17.04.2026 · 09.06.2026, delegate-approved, 56/56/40 hours against a 76-hour fortnight, each recording "Continuous Shift Worker". ■■■ **The delegate on all three is Scott Hughes — who signed the request of 31 July 2026 asking whether Mr Shepherd can fulfil the inherent requirements "without restrictions or modifications to duties".** ■ That point is NOT put to the clinician. It belongs in the covering note to the employer and in the employment track.

8. ■ **The April 2025 individual monthly statistics** — 269 to 444 calls per shift at about 29% of full time. Without them the clinician cannot address why reduced hours did not reduce intensity, and *"he kept working for fourteen months"* goes unanswered.

9. ■ **Who is performing the duties, which shifts, from when** — in writing. Currently colleague report only. Load-bearing for 3.6 and for the employer's accommodation question.

SEQUENCING

10. ■■ **Serve the AO3 role description on the Regulator as continuing disclosure BEFORE the report is commissioned.** She does not hold it — the amended List of Documents has 52 items and no role description of any kind. If the report relies on a document she has never seen, it surfaces through an expert rather than through disclosure. ■ **Serve the document, not its provenance:** no mention of the request for medical information, the exclusion, or 31 July 2026.

11. ■ **Assume everything sent to the practice is produced to the Regulator** under the notice it holds. That is why the instruction and the covering email carry no strategy, no characterisations, and nothing from the parallel tracks.

■■■ WHEN THE REPORT GOES TO MSH — CITE THEIR OWN POLICY

The adjustments the report specifies are measured against **HR Policy G3 / QH-POL-210, Reasonable Adjustment**, which MSH itself invoked in the request of 31 July 2026. ■ **Its own words, verified:**

*"The question of whether an adjustment is unreasonable or would cause 'unjustifiable hardship' is **tested against the whole organisation, not a division or unit within the organisation. The onus is on Queensland Health, as the employer, to prove an adjustment is unreasonable, not on the person to prove that it is reasonable.**"*

⇒ ■■■ **The test is not whether the Logan Switchboard roster can absorb the adjustment. It is whether Metro South Health can — and the onus of showing it cannot is theirs.** The same policy adds that failure to provide reasonable adjustment *"may constitute unlawful discrimination."*

■ **This goes in the covering note to MSH, not to the clinician.** A psychiatrist does not apply an HR policy. Question 3.6(c) is framed so that his answer — whether adjustments of that kind are ordinarily provided by large employers and by 24-hour health services — maps onto the whole-organisation test without his being asked to apply it.

WHAT THE REPORT MUST NOT DO

- Say he cannot fulfil the inherent requirements · ■ answer the accommodation question in the negative ·
- opine on working with any named person or reporting line · ■ express any view on whether management action was reasonable · ■ attribute the injury to a course of management conduct generally

rather than to the rostering and fatigue strand specifically.